PRESIDENCY UNIVERSITY
KOLKATA

ASSESSMENT OF THE SWASTHYA SAATHI SCHEME

A West Bengal Government Initiative

NAME – IVAN DAS

CLASS – UG SEMESTER 5

PAPER CODE- ECON05DSE1

ROLL NO.- ECON066

REGISTRATION NO.- 22201110066

CONTENTS

1. ***INTRODUCTION***..... 3

2. ***RESEARCH OBJECTIVE***……3

3. ***DATA***

3.1 Recruitment Criterion 3

3.2 Survey Sample 3

3.3 Data Collection 3-4

4.***PROFILE OF THE RESPONDENTS***.....4-6

5***. ENROLMENT AND AWARENESS***

5.1 Process of Enrolment 6-7

5.2 Awareness of the Benefits of the Swasthya Saathi Scheme 7

5.3 Reasons for Non-Enrolment 8

6.***HOSPITALIZATION EPISODE AND EXPERIENCE***

6.1 Hospitalization Episode 9

6.2 Experience with the Swasthya Card 9-10

6.3 Reasons for not using the card 10

7. ***FAMILY STATUS*** 11

8. ***CONCLUSION*** 11-12

9. ***APPENDIX*** 12-13

1. **INTRODUCTION:** *Swasthya Sathi*, launched in 2016 is a health insurance scheme by Government of West Bengal which provides cashless medical coverage up to Rs.5 lakh per family annually for the residents of the state. It provides smart card to each family which is led by the senior female member. Swasthya Sathi allows access to cashless treatment at empanelled hospitals for both pre-existing and new medical conditions without any family size limit. The scheme is funded by the state and it aims to reduce financial pressure during medical emergencies and improve overall health outcomes for beneficiaries.

The following are some key features of the scheme:-

- **Coverage**: The scheme provides basic health coverage for secondary and tertiary care up to Rs. 5 lakh per family per year. It covers all pre-existing diseases and conditions, and there's no cap on family size.
- **Paperless and cashless**: The scheme is paperless and cashless, and beneficiaries receive a smart card that can be used for all transactions. The smart card is valid for life and is automatically renewed each year.
- **No beneficiary contribution**: The state government pays the entire premium, so beneficiaries don't need to contribute anything.

  To apply for the Swasthya Sathi scheme, you can fill out an online application form with your personal and family details, including income information. You'll also need to upload supporting documents like identification proof, address proof, and income certificates.

2. RESEARCH OBJECTIVE:

A field questionnaire was made to collect data on the services provided by the Swasthya Sathi scheme and how it affected the general health of the beneficiaries and how satisfied were they after they enrolled in such a scheme.

3. DATA:

3.1. Data collection: In our survey, we developed a structured questionnaire addressing multiple facets of the Swasthya Sathi scheme, including awareness, enrolment procedures, hospitalization episodes, reasons for non-enrolment, and perceived usefulness of the scheme. Data were gathered from respondents in and around Howrah(22%), Kolkata(68%), their suburbs, and other nearby districts(10%) during August and September 2024.

Residential area	Freq.
Kolkata	28
Howrah and subarbs	88
Other districts	13
Total	129

3.2 Sample size: The sample size for this study consisted of 129 individuals in total from various religious,social ,gender and age groups.

3.3 Recruitment Criteria: In our study, the recruitment criterion was structured to include families in which one or more members had been hospitalized within the past 2–3 years. We prepared a questionnaire and interviews were primarily conducted with the senior female family member holding the Swasthya Sathi card. In cases where the female

member was unavailable, a male family member was permitted to participate only if he was present during the enrolment process.

4. PROFILE:

Our Sample consisted of 124 female participants and 5 male participants. The sample consisted of both people enrolled in the scheme as well as not enrolled in the scheme. The number of enrolled people were 108 and not enrolled people were 21 in number. We drew out the proportion of people enrolled and not enrolled gender-wise, religion-wise ,social group-wise, whether they hold a BPL card or not. Here is a brief overview of the stats:

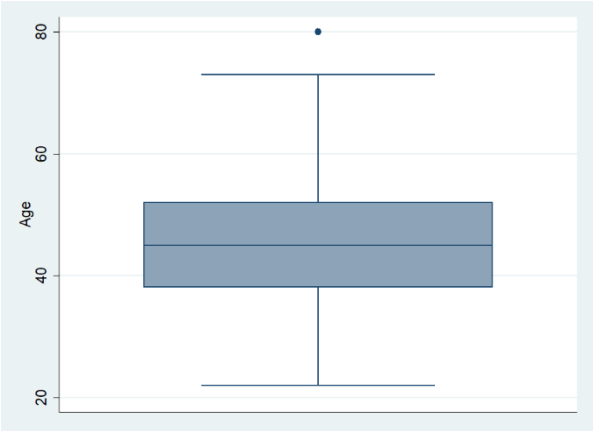
Out of the entire sample,84% of the participants were enrolled in the scheme while 16% were not.

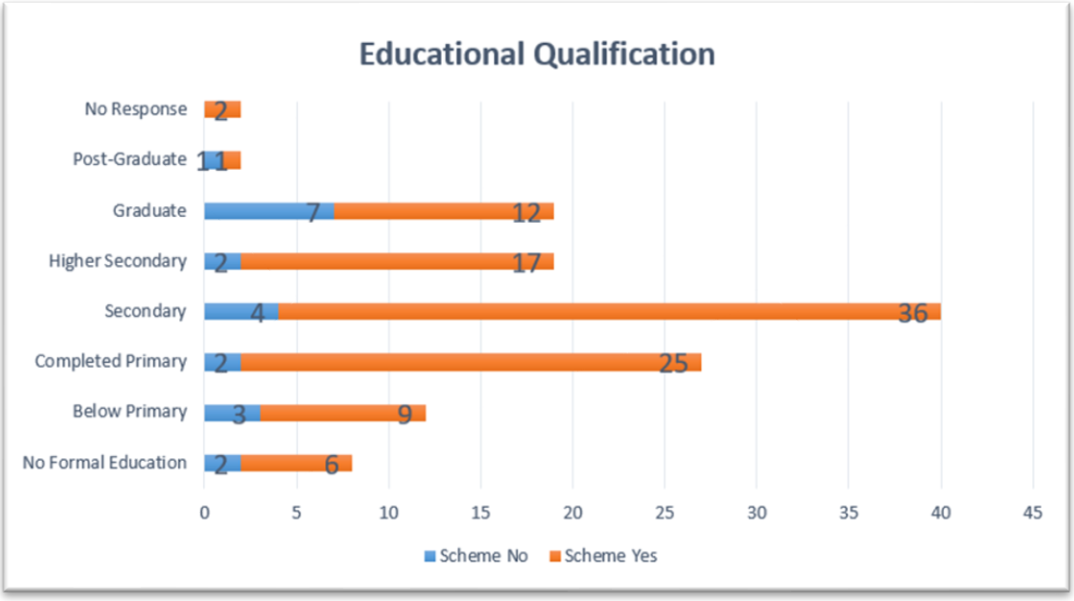
RELIGION WISE: Hindus cover about 97% of the sample whereas the Muslims cover only 3% of our sample. Amongst the Hindus about 85% of them are enrolled in the scheme and amongst the Muslims about 50% are enrolled while 50% are not.

SOCIAL GROUP WISE: Our sample reveals that Scheduled Caste (SC) individuals have the highest enrolment rate at 88%, followed closely by the General caste at 84%. In contrast, the Scheduled Tribe (ST) group has the highest non-enrolment rate, with 33% not enrolled in the scheme.

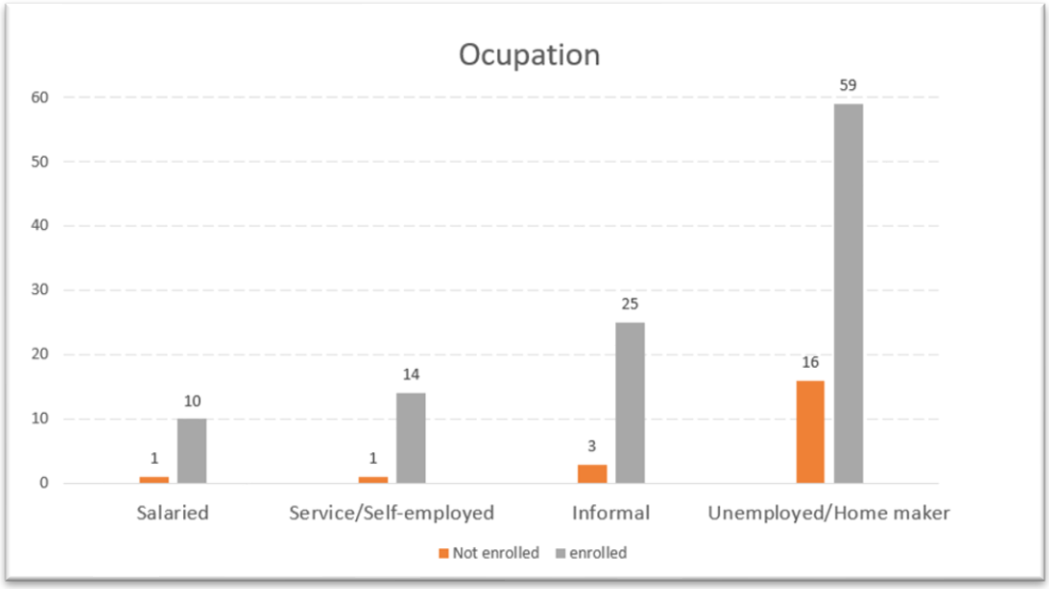
BPL CARD HOLDER: People who are below the poverty line of the nation hold the BPL card. Through our sample we get an idea that amongst the non-BPL card holders the enrolment is relatively lower than the people with BPL cards. About 95% of the BPL card holders are enrolled in the scheme where as only 77% of the non-holders are enrolled in the scheme. It is an indication that people below the poverty line might be more interested or be the targeted beneficiaries of the scheme. ***(See appendix 9.1)***

The range of ages is from approximately 20 years to 80 years. The median age is around 50 years. The majority of individuals are between 40 and 60 years old. There is only one outliers at around 80 years. These are the dots outside the whiskers.





The educational attainment of the respondents varied significantly. Approximately 6% had no formal education, while 9% had not completed primary education. About 21% had completed primary education, and one-third of the respondents had completed secondary education. Around 15% had attained higher secondary education or had graduated, and only 1.5% were postgraduates. It is most expected that the enrolment rates drop as the educational qualification of the participants improve and that is what we see here also. Almost 90% of the people with lower than higher secondary education are enrolled while graduates and post graduates make up about 50-60%. Also people with no formal education show a lesser enrolment rate too because of lack of awareness. We found out that educational qualification does not affect the enrolment rates significantly.



Our study categorizes occupation into four groups: Salaried, Service/Self-Employed, Informal, and Unemployed/Homemaker. Among these, the Service/Self-Employed group has the highest enrolment

rate in the Swasthya Sathi scheme at 93.33%, followed by the Salaried and Informal groups. Notably, among the Unemployed/Homemaker we notice the highest proportion of people not enrolled despite this group lacking employment, which may indicate a gap in outreach or access for individuals without formal income sources.

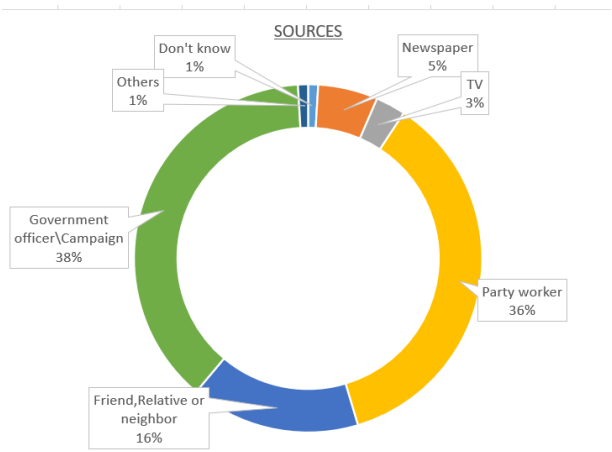
Among the 129 respondents, 39% have fewer than three family members, 33% have four family members, and 28% have five or more members. Enrolment rates vary statistically significantly with the number of family members. However, the number of aged people in the family doesn't statistically significantly affect the enrolment rates. ***(see appendix 9.4 and 9.5)***

Again, 57% of the respondents' have less than one earning member in their families and 43% of them has two or more earning members.7 After regressing the number of earning members on the enrolment rate, it was observed that the coefficients were statistically significant. Furthermore, enrolment rates vary statistically significantly with the number of earning members.

5.ENROLMENT AND AWARENESS:

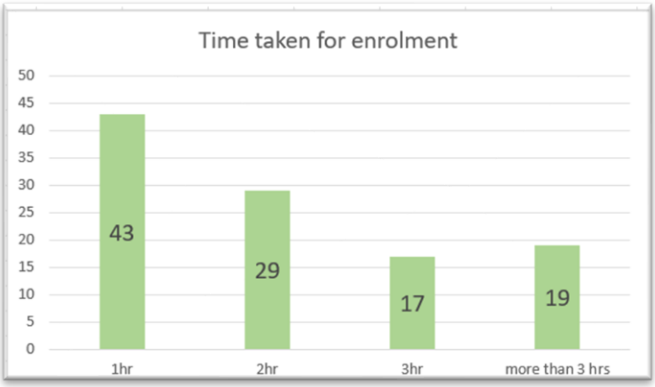
5.1 PROCESS OF ENROLMENT:

From our study, majority (about 74%) of the people reported that they became aware of the scheme through government campaigns and party workers. Other sources of information included newspapers, television, and word of mouth from friends, relatives, or neighbours.



Enrolment was mainly done through "***Duare Sarkar***" as almost 97% of the participants reported to have enrolled through this medium and the rest 3% in online mode.

We collected information about the time it took for each participant to enroll(in minutes) and we grouped the data into 4 categories:"1 hour", "between 1 and 2 hours", "between 2 and 3 hours", "more than 3 hours". We found out that about 40% of the people had their enrolment done within an hour,27% between 1 hr and 2hr, 15% between 2hr and 3hr and 18% more than 3 hrs. This in indicative of the long time duration barrier to enrolment to some extent.



We also collected data about the attitude of the husbands towards the enrolment of the scheme. Although this is a very sensitive area and we do not expect people to be completely transparent about this question due to typical social norms and patriarchy in the society but

out of responses we collected it is evident that a better support from the husband may likely be better when it comes to enrolment in such schemes.

5.2 AWARENESS OF THE BENEFITS OF THE SWASTHYA SAATHI SCHEME:

In order to assess how well the respondents are aware of the basic benefits that are offered by the scheme we set up some true and some false statements in the questionnaire related to the benefits and asked them to say "yes" or "no" or "don't know" to those.

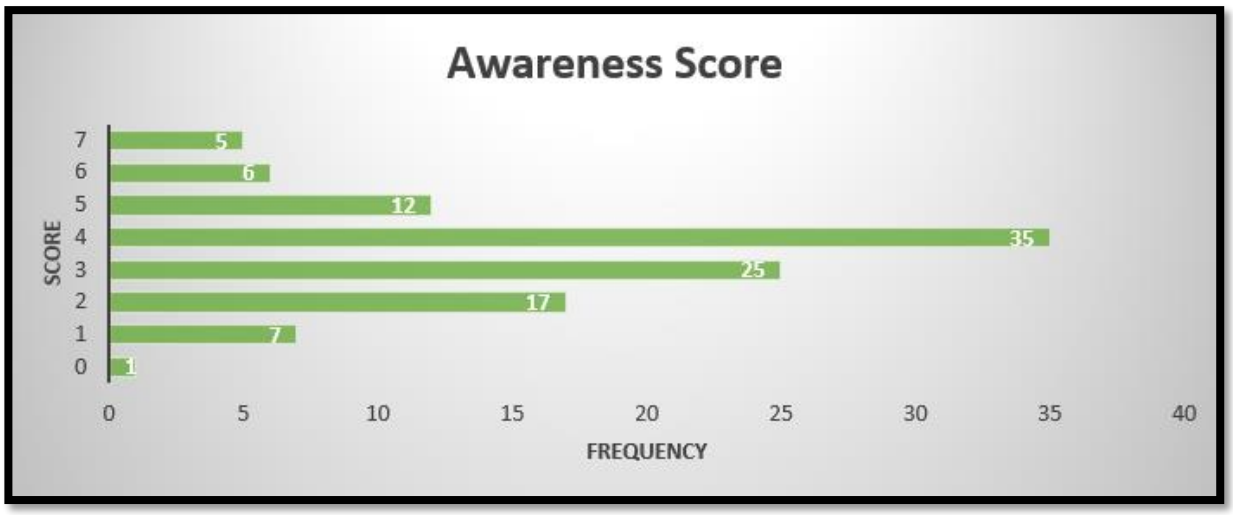
CORRECT STATEMENTS:

1. Entitled to a travelling allowance of Rs.200.
2. Costs of Emergency Care are covered.
3. Costs of Hospitalization are covered.
4. Maternity and New Born costs are covered

WRONG STATEMENTS:

1. Costs of Outpatient Department (OPD) are covered.
2. Swasthya Saathi Card can be used outside West Bengal.
3. Husband, Son or other male members of the family can also register.

We collected the responses and calculated the score for each of the respondent. The score was calculated in this manner that for every right answer they were rewarded 1 point and failure to do so earned them 0 points. This was done after the compilation of the entire data and not during the collection process.



This approach helped us evaluate the respondents' awareness and understanding of the true benefits provided by the Swasthya Saathi Scheme.
Only 5 people were able to state accurately all the benefits .From this we infer that very few people are actually aware of all the benefits of the scheme and lack the full utilisation of the efforts of the government.

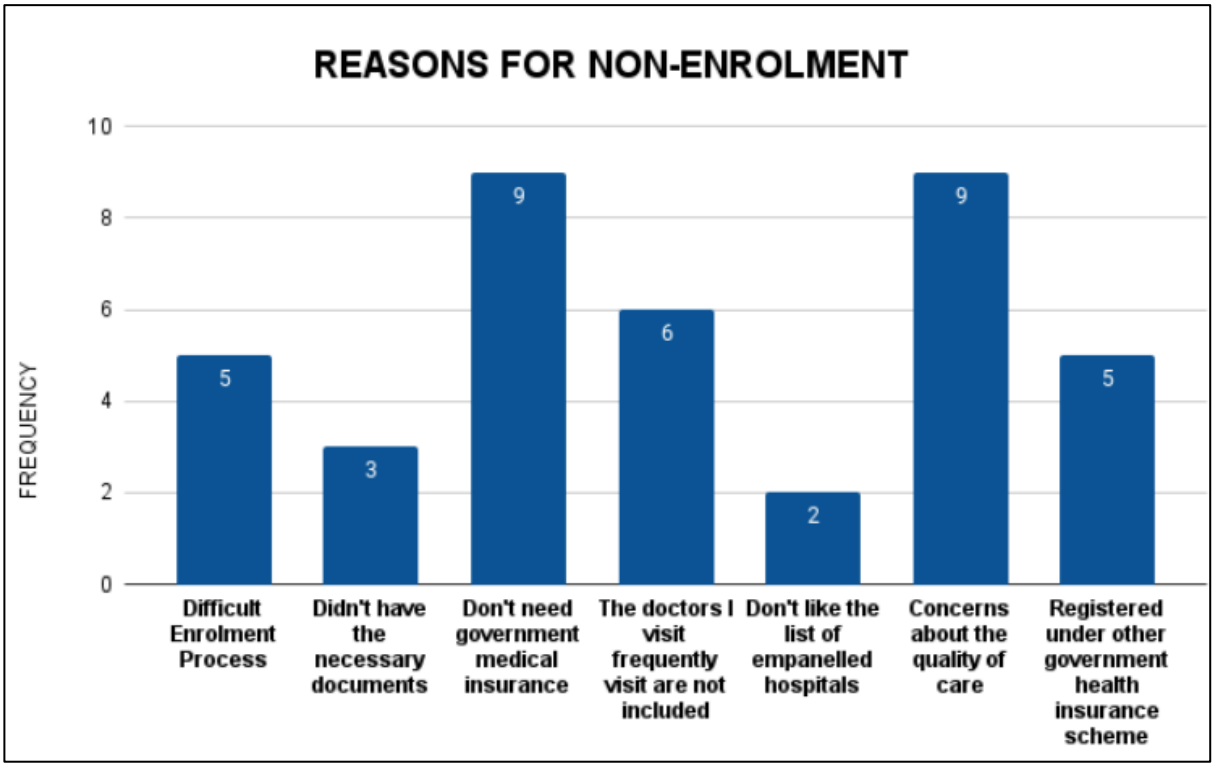
5.3 REASONS FOR NON-ENROLMENT:

The primary reasons for non-enrolment, cited by approximately 46% of respondents, were a lack of perceived need for government medical insurance and concerns about the

quality of care. This indicates a significant trust deficit in the state's ability to deliver high-quality healthcare, presenting a critical challenge for the government. To address this, efforts must be made to build public confidence in government hospitals and the Swasthya Saathi Scheme.

Additionally, around 13% of respondents reported difficulties with the enrolment process, underscoring the importance of streamlining and simplifying these procedures to ensure broader accessibility. Another 20% expressed dissatisfaction with the list of empanelled hospitals and doctors, suggesting a need for the government to expand and enhance the network of healthcare providers available under the scheme. Moreover,13% of respondents were ineligible for the Swasthya Saathi Scheme due to their registration under other government health insurance programs.

In summary, addressing these issues through targeted improvements in trust-building, process efficiency, provider network expansion, and accessible online enrolment options can significantly enhance the scheme's reach and effectiveness.

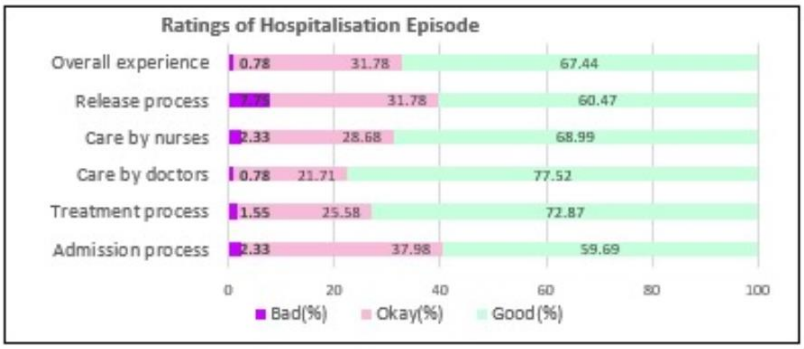


6.HOSPITALIZATION EPISODE AND EXPERIENCE:

6.1 HOSPITALIZATION EPISODE

In our study we tried to find out how many among the enrolled people actually used their cards and who did not. We found that about 54% of the respondents actually used the card. We also recorded few information about the hospitalisation episode like the time gap between the onset of symptoms and diagnosis, the time gap between diagnosis and hospitalization, delays due to a lack of available beds, and the waiting time for admission. We tried to find any noticeable difference between the two groups through Kruskal-Wallis test but we did not find any statistically significant difference between the two groups in any respect. This suggests that these factors were similar across both groups, indicating that the decision or ability to use the SS card was not influenced by these specific aspects of the healthcare experience. **(*see appendix 9.6 and 9.7*)**

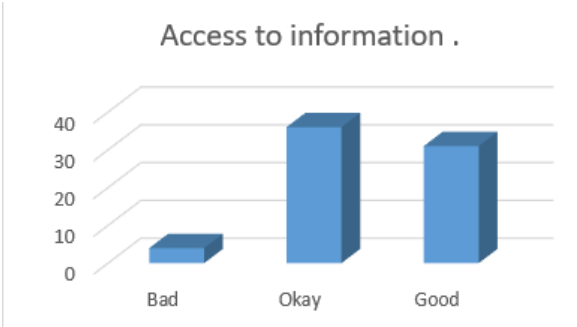
The respondents were also asked to rate their experience, in terms of the admission process, treatment process, care taken by doctors and nurses, release procedure and overall experience. There were no statistically significant variations in the ratings between the groups that used the Swasthya Saathi cards and those who did not(Kruskal-Wallis test showed no significant difference at 10%LOS)

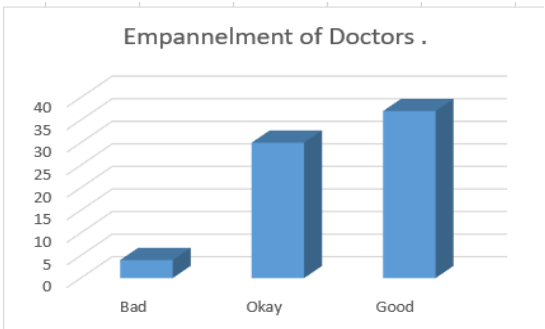


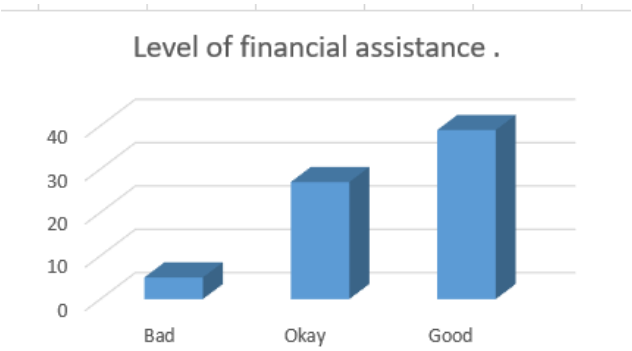
6.2 Experience with Swasthya Sathi Card:

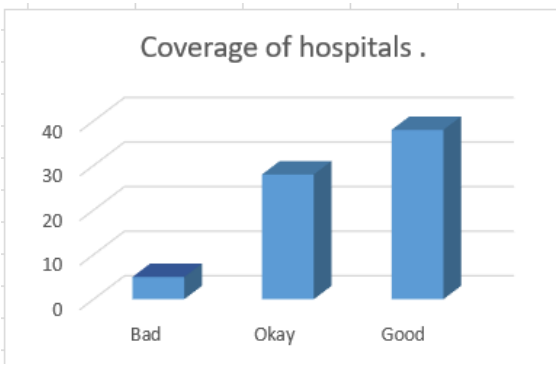
We asked the respondents to rate their satisfaction level with Swasthya Sathi scheme and many of its dimensions.

As response of the very first question that if the respondents with SSA card are satisfied with Swasthya Sathi Scheme, we found out that 90% of them are satisfied with the scheme and its benefits. they were asked to rate specific aspects, including access to information about the scheme, the coverage and availability of hospitals, the empanelment of doctors, and the level of financial assistance provided. These ratings offer insights into how well the scheme meets participants' expectations in terms of accessibility, provider choice, and financial support. By evaluating these factors, we can better understand respondents' overall satisfaction and identify areas where the scheme could be enhanced. The following bar graphs show the ratings across different aspects:



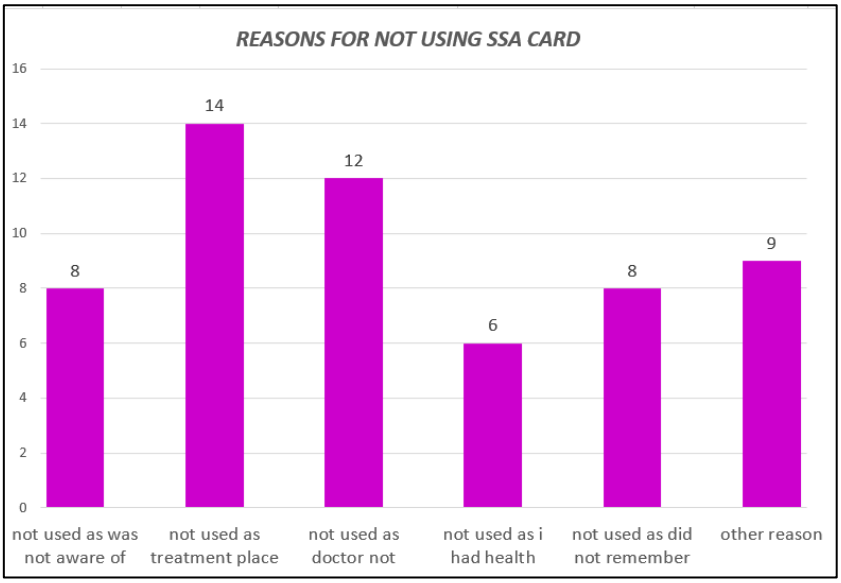






6.3 Reason for not using the card:

An analysis of the reasons behind the non-utilisation of the Swasthya sathi card revealed that the primary barriers are the limited availability empannelled treatment facilities and doctors. Additionally, some respondents indicated a lack of awareness about the card's benefits, while other mentioned simply forgetting about it. These findings highlight the need for the government to focus on expandi9ng the scheme's network by including more hospitals and healthcare providers. Enhancing public awareness about the card's advantages and the usage would also be crucial to promoting its effective utilization.



7. FAMILY STATUS:

In this section, respondents were asked about their likelihood of discussing health-related issues after the implementation of the Swasthya Saathi Scheme, using a scale ranging from "not at all likely" to "almost always." The analysis uncovered statistically significant differences between those who had used the card and those who had not.
(***see appendix 9.8 and 9.9***)

Among card users, majority rated themselves as "not at all likely" to "somewhat likely" to discuss health-related issues. In contrast, only 40% of non-card users fell within these categories. Interestingly, 29% of non-card users indicated that they were "almost always" likely to discuss health-related issues, compared to just 16% of card users. Additionally, around 18% of non-card users expressed uncertainty about their likelihood of discussing health-related issues more frequently, while only 9% of card users shared this uncertainty.(See appendix).This indicates that people with SSA card are more casual about health and emergency cost relating to health due to the coverage provided by the scheme.

Respondents were also asked whether the Swasthya Saathi card had improved accessibility to healthcare services. Among card users, 59% reported that the Swasthya Saathi card had enhanced accessibility to healthcare services since its introduction. In contrast, only 29% of non-card users shared this view. Furthermore, 39% of non-card users expressed uncertainty about whether the scheme had improved accessibility, compared to a notably lower 26% of card users who were uncertain. The analysis showed no statistically significant differences between those who had used the card and those who had not.

8.CONCLUSION

The Swasthya Sathi scheme is a transformative initiative that has significantly contributed to making healthcare accessible and affordable for a vast section of the population in West Bengal. However, its full potential can be realized only by addressing the identified barriers, improving awareness, and expanding its reach and inclusivity. As the scheme evolves, a focus on community engagement, trust-building, and infrastructure enhancement will be pivotal in ensuring that it continues to safeguard the health and well-being of all its beneficiaries.

Despite its accomplishments, several challenges persist. Awareness of the scheme’s full benefits remains limited, with only a small percentage of respondents accurately identifying all its features. Misconceptions about the scheme, such as coverage for outpatient care or its usability outside West Bengal, point to gaps in communication strategies.

Non-enrolment remains a concern, with respondents citing a lack of perceived need for government medical insurance, concerns about the quality of care, and dissatisfaction with the empanelled network of hospitals and doctors. About 20% of non-enrolled individuals expressed dissatisfaction with the list of available healthcare providers, emphasizing the need for broader and more inclusive empanelment.

Additionally, logistical barriers such as time-consuming enrollment procedures and limited outreach to specific groups like the unemployed or homemakers highlight areas for

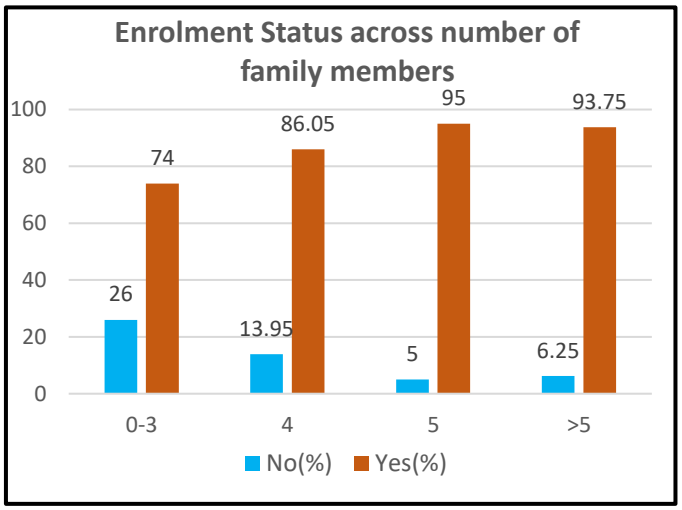
improvement. Challenges also persist in the utilization of the scheme, as only 54% of cardholders reported using their cards for hospitalization. Barriers like limited empanelled ---- treatment facilities, lack of awareness, and occasional neglect of the card's utility suggest that a portion of beneficiaries is unable to fully leverage the scheme's benefits.

APPENDIX

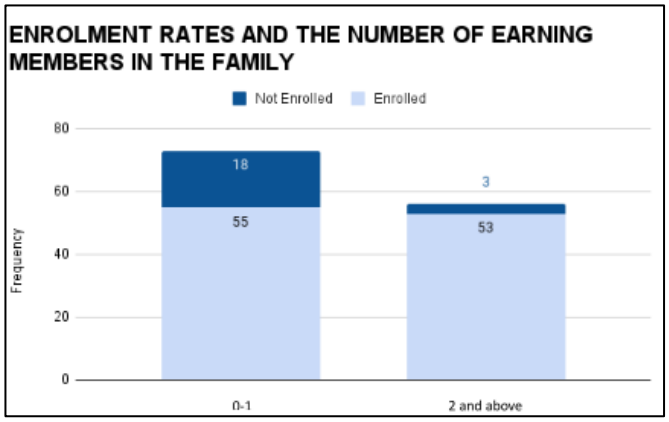
9.1

	Enrolled in SSA scheme		
Category	No(in %)	Yes(in %)	Total Freq
Gender			
Male	20%	80%	5
Female	16.13%	83.87%	124
Religion			
Hindu	15.20%	84.80%	125
Muslim	50%	50%	4
Caste			
General	16.47%	83.53%	85
Scheduled Caste (SC)	11.54%	88.46%	26
Scheduled Tribe (ST)	33.33%	66.67%	3
Other Backward Classe	16.67%	83.33%	12
No Response	33.33%	66.67%	3
BPL Card Holder			
No	22.62%	77.38%	84
Yes	5%	95%	40
No Response	0	100	5
Total	**16.28%**	**83.72%**	**129**

9.2



9.3



9.4

Probit regression model of enrolment rate on no of aged members

Probit regression	Number of obs	=	129
	LR chi2(1)	=	0
	Prob > chi2	=	1
Log likelihood = -57.274885	Pseudo R2	=	0

e1	Coef.	Std. Err.	z	P>z	[95% Conf.	Interval]
p10	-.0530334	0.1974973	-0.27	0.788	-.4401211	0
_cons	1.009558	0.1656318	6.1	0.000	.6849259	1

9.5

Probit regression model of enrolment rate on no of family members

Probit regression	Number of obs	=	129
	LR chi2(1)	=	4.39
	Prob > chi2	=	0.0362
Log likelihood = -55.115944	Pseudo R2	=	0.0383

e1	Coef.	Std. Err.	z	P>z	[95% Conf	Interval]
p9	.2180836	0.115689	1.89	0.059	-.0086619	0.444829
_cons	.1624274	0.439755	0.37	0.712	-.6994772	1.024332

9.6

Kruskal-Wallis equality-of-populations rank test

Time gap between desease and symptom

rhs	obs	rank sum
not used card	59	3565
used card	70	4820

chi-squared = 1.629 with 1 d.f.
probability = 0.2018

chi-squared with ties = 2.273 with 1 d.f.
probability = 0.1317

9.7

Kruskal-Wallis equality-of-populations rank test

Number of days admitted

rhs	obs	Rank sum
not used card	59	3425
used card	70	4960

chi-squared = 3.757 with 1 d.f.
probability = 0.0526*

chi-squared with ties = 3.782 with 1 d.f.
probability = 0.0518*

9.8

Likelihood	used used	not used card	Total
not at all likely	8.47%	27.14%	18.60%
only under very serio	6.78%	12.86%	10.08%
somewhat likely	16.95%	22.86%	20.16%
very likely	11.86%	12.86%	12.40%
almost always	27.12%	15.71%	20.93%
don't know/uncertain	28.81%	8.57%	17.83%
Total	100.00%	100.00%	100.00%

9.9

Kruskal-Wallis equality-of-populations rank test

Discussion of health related issues

rhs	obs	rank sum
not used card	59	4686
used card	70	3699

chi-squared = 16.186 with 1 d.f.
probability = 0.0001***

chi-squared with ties = 16.727 with 1 d.f.
probability = 0.0001***